

Statement of Interest — Step 1

FairBanks Medical Centre · FairBanks Community Reach · FCHIP

Addendum E: Health Foreign Assistance MOU Implementation in Uganda | Objective 4: Strengthen and Scale Digital Health Systems
Your health, our mission.



FairBanks — community health ecosystem rooted in Uganda

SOI deadline: 31 July 2026, 5:00 PM EDT | DFOP0017890 | Addendum E — Uganda

I. The Problem We Address

Uganda's five-year health transition (April 2026-December 2030) asks facilities, districts, and communities to keep HIV, TB, malaria, maternal and child health, immunisation, and outbreak services strong while shifting toward government-led, performance-based programming. That shift only works if decision-makers see what is happening at the last mile - not weeks later in paper registers.

Community Health Workers and Village Health Teams already walk households and refer patients. Too often their work never reaches eCHIS, DHIS2, facility EMRs, or district dashboards in a usable form. National digital systems then miss early fever clusters, missed antenatal visits, immunisation gaps, and stock pressure. The result is reactive care, weak accountability, and higher risk that transition gains reverse.

II. Statement of Interest

FairBanks Medical Centre and FairBanks Community Reach respectfully submit this Statement of Interest under Track 2, Objective 4 of the Uganda MOU Implementation Addendum: strengthen and scale digital health systems with full interoperability, government ownership, and sustainable operations.

We propose to pilot and then transfer a community-to-facility intelligence layer - the FairBanks Community Health Intelligence Platform (FCHIP) - that captures CHW/VHT household and visit data offline, syncs securely, maps indicators to national reporting elements, and feeds facility and district action. FCHIP is not a parallel silo. It is a bridge that makes Uganda's existing digital stack (eCHIS, DHIS2, facility EMRs, and related national platforms) more complete at community level and more useful for prevention.

III. Win-Win Value Proposition

This partnership is designed so each party gains something durable - not a one-sided software purchase.

Who gains	What they gain
Uganda / Ministry of Health	Richer last-mile data into eCHIS and DHIS2; clearer referral closure; documented handoff pathways toward government ownership and reduced dependence on parallel partner tools.
U.S. Department of State / GHSD	A Ugandan implementer with a live clinic and community network that protects essential services while proving interoperable digital scale-up under the MOU - accountable, measurable, and transferable.
Districts & facilities	Dashboards for outreach targeting, referral follow-up, and programme monitoring; GIS hotspot views that support planning without waiting for outbreaks to arrive at the ward.
Communities & CHWs/VHTs	Simple offline tools, faster support from the medical centre, and care that reaches mothers, children, older persons, and people with chronic conditions before crises.
FairBanks / FCHIP	Room to refine and document a production-grade interoperability model inside a real catchment - then share specifications, training packages, and transfer plans with MoH-aligned partners.

IV. Proposed Objectives (Objective 4)

- O1. Deploy offline-capable CHW/VHT capture aligned with eCHIS household and visit concepts across FairBanks Community Reach catchments.
- O2. Establish a secure sync and role-based access pipeline linking community records to FairBanks Medical Centre workflows and authorised district viewers.
- O3. Map community indicators to DHIS2 reporting elements and demonstrate aggregate submission pathways for MNCH, immunisation, NCD screening, and disease surveillance signals.
- O4. Deliver facility and programme dashboards plus GIS hotspot views that trigger outreach, referrals, and supply planning actions.
- O5. Produce interoperability specifications, data-governance protocols, training curricula, and a staged government-ownership transition plan suitable for full-proposal detailed design.

V. Project Approach

Anchor in a live cascade: community members identify needs; CHWs/VHTs collect and educate; FairBanks Community Reach runs outreach and home visits; FairBanks Medical Centre provides clinical care, diagnostics, pharmacy, and referrals; FCHIP is the intelligence layer that closes the data-and-feedback loop.

Build API-first, standards-aware exports - not a closed proprietary vault. Prioritise interoperability with eCHIS (household/visit concepts), DHIS2 (aggregate indicators), facility EMR encounter/referral handoff, and readiness for national warehouse feeds when schemas are confirmed.

Protect essential services during transition: structure community data around HIV/TB/malaria flags where relevant, maternal and child pathways, immunisation follow-up, and outbreak early warning so digital work reinforces - not distracts from - service continuity.

- Co-design forms and alerts with CHWs, facility staff, and local leaders; train Ugandan data stewards and developers; document every interface for MoH review.
- Start where FairBanks already operates (Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya and surrounding communities), then design for district replication using Uganda's existing CHW/VHT architecture.



Community cascade → FCHIP → national digital health systems

VI. Expected Outcomes

Outcome	What success looks like
Digital completeness	Increase structured community visit capture flowing into facility and aggregate reporting pathways within the pilot catchment.
Interoperability proof	Working mappings and demo exports for eCHIS-aligned records and DHIS2 aggregates, with open documentation for MoH and partners.
Service continuity signals	Earlier alerts for missed ANC, immunisation gaps, fever clusters, and referral drop-offs - linked to CHW follow-up and clinic action.
Accountability	Role-based audit trails, data-quality checks, and programme dashboards usable for performance-oriented district conversations.
Ownership readiness	Handoff package: source documentation, SOPs, training, hosting options, and a costed path to transfer recurrent operations toward domestic systems.

VII. Organizational Capacity

- Ugandan social enterprise with a functioning medical centre - clinical care, diagnostics, pharmacy, and referral pathways already in daily use.
- Active FairBanks Community Reach programmes: maternal and child health, Gericare, chronic disease screening, school and corporate health outreach.
- Established CHW/VHT relationships in Kampala-area communities named above - real users, not a greenfield pilot.
- Digital health records foundation and pharmacy dispensing data already supporting facility operations.
- Research orientation and partnership practice - evidence, ethics, and skills development sit inside the FairBanks community health model.

VIII. Interoperability Approach

Platform	Integration pathway
eCHIS	Household registration, visit logs, referral flags via structured mobile capture
DHIS2	Aggregate exports for immunisation, ANC, NCD screening, disease surveillance
Facility EMR	Referral handoff and encounter linkage at FairBanks Medical Centre
National warehouse / related systems	Standardised JSON/API feeds when national schema is confirmed

IX. Alignment with MOU Guiding Principles

Principle	How FairBanks responds
Sustainability & country ownership	Design for MoH alignment and staged transfer of operations
Protect essential services	Digital workflows reinforce HIV, TB, malaria, MNCH, immunisation
Data-driven accountability	Quality data for decisions - not parallel reporting burden
Integrated, people-centred care	One community-to-facility loop; less fragmentation
Local partnerships	Government, CHWs/VHTs, CBOs, academic and NGO partners
Evidence-based programming	Pilot in live catchments; measure; refine before scale

X. Geographic Focus

- Primary pilot: FairBanks catchment - Kampala-area communities with live CHW operations.
- Scale path: architecture designed for district replication wherever CHW/VHT systems operate.
- MOU fit: Ugandan organisation implementing under national health priorities and digital stack.

XI. Sustainability & Local Ownership

- Core clinic and outreach continue as FairBanks operations - the grant accelerates interoperable intelligence and transfer, not basic organisational survival.
- Diversified sustainability pathways after transfer: facility use, district deployments, NGO programme monitoring, partner APIs, and training packages.
- Local capacity: train Ugandan developers, CHW supervisors, and data stewards; document systems for government handoff.
- Community ownership: CHWs and leaders co-design forms and alerts - technology serves the cascade, not the other way around.

XII. Partnership Landscape

- Ministry of Health digital health and community health strategy alignment
- District health teams for aggregate reporting validation and readiness dialogue
- CHWs, VHTs, and community leaders as primary data partners
- Academic partners for evaluation and interoperability testing
- NGOs and CBOs already collaborating with FairBanks Community Reach

XIII. Readiness for Full Proposal

This Statement of Interest confirms FairBanks' intent to advance Objective 4 with a community-rooted, interoperable, government-aligned digital health proposal. We seek invitation to Step 2 (full proposal), where we will detail workplans, budgets, M&E, partnership letters, and a clear transition of staffing and recurrent costs toward sustainable Ugandan ownership.

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